RESIDENT INTAKE & PRE-ADMISSION PATHWAY CLINICAL SOP

7-Stage Pre-Admission Assessment, Multi-Disciplinary Matching, Zero-Void Commissioning & 28-Day Review Protocol

Operating Provider	ADOV Care Group LIMITED	**Protocol Ownership**	Registered Manager & Nominated Individual
Commissioning Partners	Luton Borough Council Adult Social Care & BLMK ICB Continuing Healthcare	**Clinical Model**	Positive Behaviour Support (PBS) & Active Support Framework
Target Service Cohort	Adults (18–64) with Autism, Severe Learning Disabilities & Step-Down Rehabilitation	**SOP Version**	Version 2.1 — Publication Edition (October 2026)
Operational Campus	78 & 80 Marston Gardens, Luton, LU2 7DY (8 Registered Beds)		

7 Stages
Admission Protocol
From desktop triage to 28-day review

Zero Voids
Commissioning Goal
100% planned occupancy retention

4–6 Weeks
Transition Window
Graduated familiarisation visits

100% PBS
Clinical Basis
Zero restraint, positive de-escalation

3-Way Match
Household Synergy
Sensory, behavioral & peer compatibility

1.0 OPERATIONAL PURPOSE, CLINICAL PHILOSOPHY & SCOPE

This Standard Operating Procedure (SOP) defines the mandatory multi-stage intake pathway for admitting residents into ADOV Care Group's supported living homes at 78 and 80 Marston Gardens, Luton. The primary clinical objective is to eliminate placement breakdown—which causes immense emotional distress to vulnerable individuals and costs commissioning authorities upwards of £20,000 per breakdown—through meticulous pre-admission compatibility matching, sensory environmental auditing, and graduated familiarisation transitions. Simultaneously, the pathway provides Luton Borough Council Adult Social Care and NHS BLMK ICB with a predictable, zero-void commissioning mechanism.

Figure 8.1: The 7-Stage Resident Pre-Admission & Community Transition Pathway

STAGE 1: REFERRAL TRIAGE (DAY 1)
Receive referral from Luton Council DPS (AT1115). Desktop suitability triage within 24h.

STAGE 2: CLINICAL ASSESSMENT (DAYS 2–5)
Face-to-face assessment by Registered Manager; review ABC charts, PBS baseline & sensory diet.

STAGE 3: PEER COMPATIBILITY (DAYS 6–8)
Evaluate household dynamic between 78 (5 beds) and 80 (3 beds); eliminate sensory clashes.

STAGE 4: FUNDING AGREEMENT (DAYS 9–14)
Finalise Individual Placement Agreement (IPA) covering core care, 1:1 hours, and housing benefit.

STAGE 5: GRADUATED TRANSITION (WEEKS 3–5)
4-step familiarisation: neutral meet, 2h tea visit, full-day visit, overnight stay.

STAGE 6: ADMISSION DAY (WEEK 6)
Tenancy agreement executed; MAR charts initiated; personal belongings unpack; keyworker assigned.

STAGE 7: CLINICAL REVIEW (DAY 28)
Multi-disciplinary 72h and 28-day statutory reviews with Care Manager, family, and resident.

2.0 STAGE-BY-STAGE OPERATIONAL PROTOCOLS & GOVERNANCE GATES

Each stage of the intake pathway enforces strict governance gates. A referral cannot advance to the subsequent stage without explicit written sign-off by the designated clinical lead.

Table 8.1: Intake Pathway Governance Gates, Documentation & Sign-Off Matrix

Pathway Stage	Operational Lead	Target Timeline	Mandatory Evidence / Documentation	Sign-Off Gate Threshold
Stage 1: Referral Triage	Brokerage Liaison	Hours 0–24	Council Care Act assessment, social history, risk summary	Basic eligibility criteria satisfied; bed vacancy confirmed
Stage 2: Clinical Assessment	Registered Manager	Days 2–5	Functional Behaviour Assessment, sensory audit, medical history	RM verifies provider can safely meet all assessed needs

Table 8.1: Intake Pathway Governance Gates, Documentation & Sign-Off Matrix (Continued)

Pathway Stage	Operational Lead	Target Timeline	Mandatory Evidence / Documentation	Sign-Off Gate Threshold
Stage 3: Peer Compatibility	House Team Leader	Days 6–8	Household sensory compatibility matrix; tenant consultation notes	Zero sensory or behavioral conflict with existing tenants
Stage 4: Funding Agreement	Finance Director	Days 9–14	Agreed IPA, Section 117 / CHC contract, Exempt Housing Benefit form	Written funding authorization from Council / ICB received
Stage 5: Transition Visits	Allocated Keyworker	Weeks 3–5	Visit observation logs, sensory response charts, resident feedback	Resident expresses positive desire; family & advocate support
Stage 6: Formal Admission	Registered Manager	Day 35–42	Signed AST tenancy, Care Contract, initial MAR chart, PEEP plan	CQC Regulation 19 vetting complete; bedroom ready
Stage 7: 28-Day Review	Multi-Disciplinary Team	Day 63–70	28-day statutory clinical review report; goal achievement summary	Placement formally confirmed as permanent by all parties

3.0 HOUSEHOLD MATCHING & SENSORY ENVIRONMENTAL SYNERGY

The unique campus structure of ADOV Care Group—offering two adjacent properties with distinct environmental profiles—provides an extraordinary operational advantage in resident matching:

Campus Allocation & Environmental Matching Architecture

78 MARSTON GARDENS (ACTIVE DYNAMIC)

5 single bedrooms. Designed for residents who enjoy active domestic engagement, communal cooking, and structured group activities in the large living room and 100ft garden.

80 MARSTON GARDENS (CALM SENSORY)

3 large single bedrooms. Designed for residents with high sensory sensitivity who require a low-arousal, ultra-quiet domestic environment with minimal stimuli.

SENSORY CLASHING PREVENTION

Tenants with vocal tics or auditory triggers are never placed in the same building. The two properties allow precise clinical separation while sharing campus infrastructure.

EMERGENCY DE-ESCALATION BUFFER

In the rare event of severe temporary friction, the dual-house campus allows short-term respite relocation across gardens without disrupting school or day-care rotas.